12. THE HEART : DIASTOLE & SYSTOLE

DURATION : 05:06 STUDY SHEET

COURSE SUMMARY

This video explores the essential dynamics of the heart's systolic and diastolic movements, focusing on their impact on bodily energy and emotions. You will learn to observe the heart's vertical and horizontal orientations, which can reveal crucial aspects of your patients' emotional health. The concepts of ventricular looping and interaction with other organ systems like the liver and lungs show how the heart maintains a regulatory pressure within its fascial environment. Practical techniques for manual assessment of the fascial skin and movements allow these notions to be applied in a therapeutic setting, while also revealing the link between the heart and the diaphragm for proper rhythmic and metabolic function.

13 - THE HEART: DIASTOLE & SYSTOLE

The **heart** stores **energy** in a **fascial** environment, thereby supporting its **electricity**, in both its ascending and descending phases.

There is an **innate predominance** in some people, who may manifest a **systolic** or **diastolic** function. A balance between these two functions is normally required. The heart repeats its embryonic movement quite directly.

HEART MOVEMENTS AND EMOTIONAL IMPLICATIONS

A specific movement can be felt. For example, people who are too oriented towards a **systolic** movement are often searching, but can also quickly become exhausted. Conversely, others start from this point and constantly seek a lower fulcrum.

VENTRICULAR LOOPING AND CARDIAC PHASES

The heart performs a characteristic **looping**, where the **ventricular** system, initially superior, becomes inferior, with a crucial fulcrum between the **atria** and the **ventricles**.

- **During systole:**

* The heart performs an **upward movement**.

* It performs a **supination** movement.

* It tends to **verticalise**.

* It performs an **untwisting**.

It is at this moment that the **aorta** opens to receive all the blood, causing **congestion**.

- **During diastole (descent):**

* All the blood from the aorta is ejected into the system.

It is sometimes easy to confuse the systolic movement with the descending movement. Systole is the moment of **ejection**. When the heart descends again, it **refills**.

When the volume is ejected, the pressure in the aortic arch system increases. At the moment of descent, the heart fills while ejecting the aorta.

HEART ORIENTATION: VERTICALISATION VS. HORIZONTALISATION

Do you have the impression that the heart tends to **verticalise** or **horizontalise**? Does it verticalise and ascend, or horizontalise and descend?

FULCRUMS FOR OBSERVATION

We will take three fulcrums:

1. At the level of the **aortic space** (second intercostal space).

2. At the level of the **pulmonary space**.

3. The **hypothenar eminence** will be placed on the apex of the heart.

THE HEART'S INFLUENTIAL SYSTEMS

The heart reacts according to two major systems that are very close to it and tends to help them as much as possible:

- The **liver**
- The **stomach**
- The **lungs**
- The **kidneys**

It always seeks to maintain this pressure in its environment. The heart is a **volume regulator**. The heart should be considered as a large artery extending with the aorta, like an **octopus**.

SUPINATION AND PRONATION MOVEMENTS

We will have a **supination** movement, then a **pronation** movement.

MANUAL ASSESSMENT AND FASCIAL REACTIVITY

We will observe the environment and how it is arranged. Then, we will place our hands at the level of the liver and observe its reaction on a **fascial** plane.

The aorta, if you observe it well anatomically, actually starts quite far back, which makes it difficult to visualise. However, it shares a common fascial environment with the **bronchi** and the **oesophagus**, which insert at the level of a vertebra often in adaptation.

RHYTHMIC ZONES AND DIAPHRAGMATIC RELATIONSHIP

There is:

- A **neurosensory** zone.
- A **rhythmic** zone.
- A **metabolic** zone.
- A **urogenital** zone.

For good movement, the heart must have a good relationship with the **diaphragm**. There is a direct relationship via the fascial continuity of the arch and the aorta.

Remember the very deep passage of the aorta, where the **diaphragmatic crura** insert on either side. To work on the heart, it is essential that the diaphragm is **open** and **supple**. Do not forget the other important passage at the level of the oesophagus.

PRACTICAL APPLICATION: FOLLOWING FASCIAL MOVEMENTS

We have studied the **embryonic pre-pattern fulcrum** which is positioned at the level of the primitive streak, at the level of the coccyx, with the arrival point.

Now, let's place the hand at the level of the heart and let it follow the fascial movement. The tissue, if you are attentive, will indicate whether the heart leans more towards systole or diastole.

Is it my liver reacting? Is it in relation to my stomach? Or is it in relation to my kidneys? (Three levels). Then, we will observe if this can rebalance in relation to this observation.